NCE Bonus Mini Practice Exam

50 new questions in the 2027 three-option format, with full answer rationales

Free Reader Bonus #1 for NCE Exam Prep 2026–2027

Keystone Test Prep Team

NCE Bonus Mini Practice Exam

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All questions in this bonus are original and were written for readers of NCE Exam Prep 2026–2027. They are not actual NCE items, and they do not repeat the questions in the book. The client stories are fictional.

How to Use This Exam

This mini exam gives you 50 fresh questions after you have worked through the two full-length practice exams in NCE Exam Prep 2026–2027. Every question uses the three-option format of the 2027 NCE, and the questions follow the six 2027 domains in roughly the same proportions as the real exam.

Take it as a timed checkpoint. On the 2027 NCE, you have 225 minutes for 170 questions, or about 79 seconds per question. At that pace, 50 questions take about 66 minutes. Set a timer, sit somewhere quiet, and answer every question before you check a single answer.

- **Predict, then look.** Read the stem, decide what a good answer would be, and only then read the options.
- **Eliminate first.** With three options, ruling out one wrong answer leaves you with a coin flip. Ruling out two leaves you with the answer.
- **Mark your guesses.** Put a small G next to any answer you guessed. A correct guess is still a weak spot.
- **Score by domain.** Use the domain map after the answer key to see which parts of the exam need more work, then add every miss and guess to your weak-spot tracker.

Mini Practice Exam

1. A client says, "My manager criticized my report in front of the whole team, and I just froze." Which response is a reflection of feeling?

A. "Why didn't you speak up for yourself in the meeting?"

B. "You felt embarrassed and caught off guard in front of everyone."

C. "Tell me more about your manager."

2. Scores on a self-esteem inventory have a mean of 80 and a standard deviation of 5. A client scores 90. What is the client's z-score?

A. 1.0

B. 10.0

C. 2.0

3. In a state with a mandatory duty-to-protect law, a client describes a specific, credible plan to seriously harm his former partner, whom he names. What should the counselor do?

A. Take the protective steps the state law requires, such as warning the partner or notifying law enforcement, and document the actions

B. Keep the statement confidential, because privilege belongs to the client

C. Wait until the next session to see whether the client still feels the same way

4. A newly licensed counselor is asked by her agency to lead a play therapy group for young children next week. She has no training in play therapy. What is the most appropriate response?

A. Lead the group, because her license allows her to work with any population

B. Lead the group after reading a book on play therapy over the weekend

C. Explain that she needs training and supervision in play therapy before leading the group, and help the agency consider other options

5. Which treatment plan objective is written most effectively?

A. Client will use a paced-breathing skill in at least 4 of 5 stressful work situations per week, as recorded in a daily log, within six weeks

B. Client will feel less anxious at work

C. Client will develop a better understanding of anxiety

6. A counselor asks, "On a scale from 0 to 10, where 10 means things are as good as they could be, where are you today? And what would it take to move up just one point?" Which approach is the counselor using?

A. Narrative therapy

B. Reality therapy

C. Solution-focused brief therapy

7. An attorney in a custody dispute asks a counselor to testify about a current client and to give her expert opinion on which parent should receive custody. What is the most appropriate response?

A. Agree to give the custody opinion, because she knows the family better than anyone else

B. Decline to act as a custody evaluator or expert, and limit any required testimony to her role as the treating counselor

C. Write a custody recommendation for the court instead of testifying in person

8. A counselor working with a 7-year-old considers how the long, unpredictable shifts at his mother's workplace affect the boy, even though he never goes there. In Bronfenbrenner's ecological model, the mother's workplace is part of the child's

A. exosystem

B. microsystem

C. macrosystem

9. A team that includes a 14-year-old, his parents, his teacher, his probation officer, and his counselor meets regularly to build one family-driven plan that coordinates services across all of these systems. What is this approach called?

A. Partial hospitalization

B. Wraparound

C. Utilization review

10. During supervision, a counselor-in-training avoids discussing a client's grief. The supervisor learns that the supervisee's mother died recently. In Bernard's discrimination model, which focus area does this concern most directly involve?

A. Intervention

B. Conceptualization

C. Personalization

11. A client says, "I absolutely must get every question right on my licensing exam, or I'm a complete failure." A counselor using rational emotive behavior therapy would most likely

A. dispute the irrational demand and the global self-rating

B. ask for the client's earliest childhood memory

C. invite the client to speak to an empty chair

12. A counselor wants to know whether all the items on a new anxiety inventory measure the same construct. Which statistic best answers this question?

A. A test-retest correlation

B. Cronbach's alpha

C. Cohen's kappa

13. Three years after ending counseling, a counselor runs into a former client at a community event. Both are single, and the counselor would like to start dating the former client. According to the ACA Code of Ethics, what applies?

A. The relationship is permitted, because counseling has ended

B. The relationship is permitted if the former client starts it

C. The relationship is prohibited, because the Code bars romantic relationships with former clients for five years

14. A client with an alcohol use disorder has stable housing, no risk of withdrawal, strong motivation, and a full-time job. Weekly sessions have not been enough to help him stay sober. Which level of care is most appropriate?

A. Medically managed inpatient withdrawal management

B. An intensive outpatient program

C. Long-term residential treatment

15. A family counselor notices that a mother and her teenage son regularly side against the father. The counselor asks the parents to sit together, has them discuss a curfew while the son waits, and blocks the son from interrupting. Which approach is the counselor using?

A. Structural family therapy

B. Bowen family systems therapy

C. Strategic family therapy

16. A principal asks a school counselor to help two teachers manage a student who disrupts class. The counselor meets only with the teachers, helping them understand the student's behavior and refine their own classroom responses. In Caplan's model, this is

A. consultee-centered case consultation

B. client-centered case consultation

C. program-centered administrative consultation

17. A new depression screening tool correctly identifies 90 percent of people who actually have depression. This figure describes the tool's

A. specificity

B. predictive validity

C. sensitivity

18. A 15-year-old client's parents ask the counselor to tell them everything their son says in sessions. What is the best way for the counselor to handle confidentiality with minors?

A. Tell the parents they have no right to any information

B. Balance the minor's privacy with the parents' legal rights and state law, ideally after agreeing with both on the limits of confidentiality at the start of counseling

C. Share everything the client says, word for word, because parents hold the legal rights

19. A counselor writes, "Client reports sleeping about four hours a night and waking with racing thoughts." In a SOAP note, where does this statement belong?

A. Assessment

B. Objective

C. Subjective

20. In the second month of a counseling group, members start challenging the leader and criticizing one another. What is the leader's best response?

A. Ignore the conflict so the group stays positive

B. Name the conflict openly, invite members to talk about it, and model nondefensive responses

C. End the group early, because it is not working

21. A client says, "I know my drinking is hurting my family. I'm going to cut back starting next month, and I've already looked up a few support group meetings." Which stage of change best fits this client?

A. Contemplation

B. Action

C. Preparation

22. A client insists, despite clear evidence to the contrary, that his neighbors have placed a device in his wall to read his thoughts. On a mental status examination, where is this recorded?

A. Thought content

B. Thought process

C. Perception

23. A counselor always calls clients back when she says she will and keeps every commitment she makes about scheduling. Which ethical principle does this behavior most clearly reflect?

A. Fidelity

B. Veracity

C. Justice

24. A counselor feels unusually angry and protective while a teenage client describes conflict with her strict father. The counselor's own father was harsh and controlling. What is the best first step?

A. Refer the client to another counselor right away

B. Explore the reaction in supervision or consultation to manage the countertransference

C. Share her own story with the client to build rapport

25. In a recovery group, a member who has been sober for two years describes how he got through his first month. Several newer members say that, for the first time, they believe recovery might be possible for them. Which therapeutic factor are the newer members experiencing?

A. Altruism

B. Catharsis

C. Instillation of hope

26. A busy counselor usually writes her progress notes about two weeks after each session. What is the best practice?

A. Complete each note promptly after the session, while the details are accurate

B. Write notes only when an insurer requests them

C. Keep brief notes in a personal notebook at home and type them up later

27. A client describes four days last spring when she slept three hours a night, felt unusually energetic, and started several projects at once. Coworkers noticed the change, but she kept working and was never hospitalized. She also had a major depressive episode two years ago. Which diagnosis best fits?

A. Bipolar I disorder

B. Bipolar II disorder

C. Cyclothymic disorder

28. A counselor's religious beliefs conflict with a client's goal of strengthening her same-sex relationship. According to the ACA Code of Ethics, what should the counselor do?

A. Refer the client because of the values conflict

B. Gently encourage the client to reconsider her goals

C. Continue working toward the client's goals, avoid imposing personal values, and seek supervision or training as needed

29. A counselor will leave her agency in six weeks. What is the best way to handle her current clients?

A. Tell clients early, discuss their options, and arrange transfers, using warm handoffs where possible

B. Tell clients at their final session

C. Leave all transfer decisions to the agency without discussing them with clients

30. A client whose Holland code is strongly Social works alone all day as a data-entry clerk and feels drained and unfulfilled. In Holland's theory, her situation reflects low

A. differentiation

B. congruence

C. consistency

31. In the second session, a White counselor says to a Black client, "I'm aware that we come from different racial backgrounds. I'm wondering how that feels for you as we talk about what's happening at your job." What is the counselor doing?

A. Using self-disclosure to build rapport

B. Imposing personal values

C. Broaching

32. Under HIPAA, which statement about psychotherapy notes is accurate?

A. They are the same as progress notes and are part of the regular medical record

B. They are kept separate from the medical record and receive additional protection

C. Insurers may obtain them whenever they request them

33. The family of an 82-year-old man reports that over the past two days he has become confused, with attention that comes and goes and worsens at night. What should the counselor do first?

A. Begin cognitive behavioral therapy for late-life depression

B. Schedule a neuropsychological evaluation in six months

C. Arrange an urgent medical evaluation for possible delirium

34. A client started an SSRI antidepressant two weeks ago. She says she feels no better yet and plans to stop taking it tomorrow. What is the counselor's best response?

A. Agree that she should stop, because the medication is clearly not working

B. Explain that antidepressants often take several weeks to show full benefit, and encourage her to talk with her prescriber before making any change

C. Suggest that she ask her doctor to switch her to a benzodiazepine instead

35. A counselor is using Roberts' seven-stage crisis intervention model with a client in acute distress. What is the first stage?

A. Developing and formulating an action plan

B. Arranging follow-up

C. Planning and conducting a thorough biopsychosocial and lethality assessment

36. A counselor records a client's daily panic attacks for two weeks without treatment, introduces an intervention, withdraws it, and then reintroduces it. Which research design is this?

A. AB design

B. ABAB (reversal) design

C. Multiple-baseline design across settings

37. A client of a counselor licensed in Ohio moves to Arizona for three months and asks to continue sessions by video. What should the counselor do?

A. Check the laws of Arizona, where the client will be located, including licensure or Counseling Compact privilege, before continuing

B. Continue without change, because only the counselor's home-state license matters

C. Switch to email sessions, because licensure rules do not apply to email

38. A graduate student wants to make sure her master's program meets the most widely recognized accreditation standards for counselor education. Which organization accredits counselor education programs?

A. NBCC

B. CACREP

C. ACA

39. During a risk assessment, a client with recent suicidal thoughts mentions that her late husband's handgun is still in her bedside drawer. What is the counselor's best step?

A. Work with the client on ways to reduce access to the firearm during this period, such as storing it with a trusted person

B. Avoid the topic, because firearm ownership is a personal matter

C. Tell the client she must sell the gun before her next session

40. A client says, "Part of me wants to quit smoking, but it's the only way I know how to relax." Which response best reflects motivational interviewing?

A. "You really need to quit. Smoking is going to make you sick."

B. "On one hand, smoking helps you relax, and on the other hand, you're thinking about quitting."

C. "Why haven't you quit already?"

41. A 9-year-old client tells her school counselor that her father hit her with a belt last night, and the counselor sees bruises on her arm. What should the counselor do?

A. Investigate further to confirm the abuse before taking action

B. Call the father to hear his side of the story first

C. Report the suspected abuse to child protective services as required by law

42. After eight sessions, a client's scores on a weekly outcome measure show no improvement. What should the counselor do?

A. Continue the current plan unchanged, because change takes time

B. End counseling, because it is not working

C. Review progress with the client and revise the goals or interventions together

43. Which CPT code describes a psychiatric diagnostic evaluation?

A. 90791

B. 90834

C. 90853

44. Which feature best distinguishes clinical supervision from consultation?

A. Supervision is evaluative and hierarchical, while consultation is voluntary and nonhierarchical

B. Consultation includes a gatekeeping role, while supervision does not

C. Supervision focuses on one case at a time, while consultation always covers the whole caseload

45. A licensing board compares each candidate's score with a fixed passing standard rather than with the scores of other candidates. This approach is

A. norm-referenced

B. ipsative

C. criterion-referenced

46. A counselor is treating a client with PTSD after a car crash using prolonged exposure. Which pair of techniques is central to this approach?

A. Imaginal exposure and in vivo exposure

B. Bilateral stimulation and resource installation

C. Externalizing and re-authoring

47. A counselor learns that a colleague is billing insurance for sessions that never took place. When the counselor raises it privately, the colleague dismisses the concern. What should the counselor do next?

A. Report the conduct to the appropriate licensing board or ethics committee

B. Take no further action, because informal resolution was attempted

C. Contact the colleague's clients to warn them

48. A client is about to finish an intensive outpatient program. Which step best supports continuity of care?

A. Closing the record without contacting the next provider

B. An aftercare plan with a scheduled follow-up appointment and relapse prevention supports

C. A general recommendation to "keep up the good work"

49. A counselor at a primary care clinic walks a patient down the hall and personally introduces her to the clinic's substance use specialist. This is an example of

A. a cold referral

B. consultee-centered consultation

C. a warm handoff

50. A counselor notices that she checks work email late every night and dreads Monday mornings. How does the ACA Code of Ethics frame self-care?

A. As a personal choice with no professional implications

B. As a professional responsibility, because counselor impairment can harm clients

C. As important only after a client files a complaint

Answer Key and Rationales

1. B (Domain 4). A reflection of feeling names the emotion underneath the client's words. B captures embarrassment and shock. A is a "why" question that can sound judgmental and puts the client on the defensive. C is an open invitation, but it shifts the focus to the manager rather than reflecting the client's feelings.

2. C (Domain 2). A z-score equals the raw score minus the mean, divided by the standard deviation: (90 − 80) ÷ 5 = 2.0. A would require a score of 85. B is the raw distance from the mean, not a standard score.

3. A (Domain 6). A serious threat against a reasonably identifiable person triggers the duty to protect where state law requires it. The counselor follows the steps the law specifies and documents them. B confuses privilege, which applies to court testimony, with the duty to protect. C delays action when a third party is at risk.

4. C (Domain 1). Counselors practice only within the boundaries of their education, training, and supervised experience. New specialties require training and supervision first. A misunderstands what a license permits. B treats a weekend of reading as competence, which it is not.

5. A (Domain 3). A good objective is specific, measurable, achievable, realistic, and time-bound. A names the behavior, how often, how it will be measured, and by when. B and C are vague and cannot be measured.

6. C (Domain 4). Scaling questions are a signature technique of solution-focused brief therapy, developed by Steve de Shazer and Insoo Kim Berg. They track progress and focus on small, achievable next steps. A focuses on externalizing problems and re-authoring stories. B uses the WDEP system to examine wants, behavior, and plans.

7. B (Domain 6). A treating counselor is a fact witness who can describe direct observations. Giving a custody opinion is a forensic role, and combining treating and forensic roles creates a conflict that can harm the client and the counseling relationship. A combines the two roles. C still offers a forensic opinion, just in writing.

8. A (Domain 2). In Urie Bronfenbrenner's model, the exosystem includes settings the child does not take part in directly but that still affect him, such as a parent's workplace. B includes settings where the child participates directly, such as home and school. C is the broad cultural context of values, laws, and customs.

9. B (Domain 5). Wraparound is a team-based, family-driven planning process for people with complex needs who are involved with several systems. A is a level of care offering several hours of structured treatment each day. C is a payer's review of whether services are medically necessary.

10. C (Domain 1). In Bernard's discrimination model, personalization concerns how the counselor's own feelings, history, and reactions affect the work. A recent loss that leads the supervisee to avoid a client's grief is a personalization issue. A concerns observable skills. B concerns understanding the client's situation.

11. A (Domain 4). Albert Ellis's REBT targets rigid "musts" and global self-ratings and teaches clients to dispute them. B reflects Adlerian early recollections. C is a Gestalt technique.

12. B (Domain 2). Cronbach's alpha measures internal consistency, meaning how well the items hang together around a single construct. A measures stability over time. C measures agreement between raters.

13. C (Domain 6). The ACA Code prohibits sexual or romantic relationships with former clients for five years after the last professional contact. Even after five years, the counselor must show the relationship is not exploitive. A and B ignore the five-year rule.

14. B (Domain 3). An intensive outpatient program adds structure and hours while letting the client keep his housing and job. A is for clients at risk of dangerous withdrawal. C is more restrictive than this client needs.

15. A (Domain 4). Salvador Minuchin's structural family therapy uses enactments, changes in seating, and boundary-making to strengthen the parental subsystem. B focuses on differentiation and multigenerational patterns, often with genograms. C relies on directives and strategies aimed at the problem behavior.

16. A (Domain 1). In consultee-centered case consultation, the consultant helps consultees, here the teachers, handle a specific case more effectively. B would involve the consultant assessing the student and recommending a plan. C concerns designing or running a program.

17. C (Domain 2). Sensitivity is the ability of a test to correctly identify people who have the condition. A is the ability to correctly rule out people who do not have it. B concerns how well scores predict a future outcome.

18. B (Domain 6). Parents or guardians usually hold the legal rights in counseling a minor, but counselors protect the minor's privacy as far as possible. The best practice is to agree on the limits of confidentiality with everyone at the start and to follow state law. A ignores parental rights. C ignores the minor's welfare and the trust the relationship depends on.

19. C (Domain 5). The Subjective section records what the client reports, including symptoms in the client's own terms. A contains the counselor's clinical impressions. B contains observable data, such as appearance, behavior, or test scores.

20. B (Domain 4). Conflict and challenges to the leader are normal in the transition or storming stage. A leader who names the conflict openly and responds without defensiveness helps the group build trust and move on to productive work. A leaves tension unspoken. C treats a normal developmental stage as failure.

21. C (Domain 3). In the preparation stage, a person intends to act soon, usually within the next month, and has taken small steps. A describes awareness without a plan to act soon. B involves active, observable change already under way.

22. A (Domain 2). A fixed false belief is a delusion, recorded under thought content, which covers what the client thinks about. B describes how thoughts are organized and connected. C covers sensory experiences such as hallucinations.

23. A (Domain 6). Fidelity means keeping promises and honoring commitments to clients. B means being truthful. C means treating people fairly and equitably.

24. B (Domain 1). Strong reactions rooted in the counselor's own history are countertransference. The first step is to explore them in supervision or consultation so they do not harm the work. A is premature, because many reactions can be managed. C shifts the focus to the counselor and risks a boundary problem.

25. C (Domain 4). Instillation of hope, one of Irvin Yalom's therapeutic factors, grows when members see others who have improved. A is the benefit that comes from helping others, which the experienced member, not the newer members, is more likely to gain. B is the release of strong emotion.

26. A (Domain 5). Records should be accurate and timely. Notes written soon after a session are more reliable, support continuity of care if another provider steps in, and protect both client and counselor. B is wrong, because counselors keep records whether or not anyone requests them. C puts client information at risk outside a secure record system.

27. B (Domain 2). Four days of elevated energy and reduced need for sleep that others notice, without marked impairment or hospitalization, is a hypomanic episode. A hypomanic episode plus a past major depressive episode fits bipolar II disorder. A requires a full manic episode, lasting at least a week or leading to hospitalization. C requires two years of milder mood swings that never meet full episode criteria, and this client has had a full major depressive episode.

28. C (Domain 6). The ACA Code directs counselors not to impose their personal values and prohibits referral based only on the counselor's values. Counselors seek training or supervision when values conflict with a client's goals. A is a values-based referral. B imposes the counselor's values.

29. A (Domain 3). Early notice, collaborative planning, and warm handoffs protect continuity of care and prevent abandonment. B gives clients no time to prepare. C excludes clients from decisions about their own care.

30. B (Domain 4). In John Holland's theory, congruence is the fit between a person's type and the work environment. A Social type in an isolated, data-focused job has low congruence, which predicts lower satisfaction. A refers to how clearly one type stands out in a person's profile. C refers to how closely related a person's top types are on the RIASEC hexagon.

31. C (Domain 1). Broaching is a counselor's intentional invitation to discuss cultural factors, such as race, that may matter to the client or to the counseling relationship. A shares personal information for the client's benefit, but the counselor's aim here is to open a conversation about culture. B does not apply, because the counselor imposes no beliefs or values.

32. B (Domain 6). Under HIPAA, psychotherapy notes are a counselor's separately kept notes analyzing session content, and they receive extra protection. A confuses them with progress notes. C is wrong, because insurers generally cannot demand psychotherapy notes as a condition of payment.

33. C (Domain 2). Sudden onset over hours to days, fluctuating attention, and worsening at night point to delirium, which needs urgent medical evaluation. A skips the medical emergency. B fits a gradual neurocognitive decline, not an acute change.

34. B (Domain 5). Antidepressants such as SSRIs often take several weeks to reach their full effect, and medication decisions belong to the client and her prescriber. Counselors give accurate general information and encourage communication with the prescriber. A is outside a counselor's scope, and stopping abruptly can cause problems. C is also outside the counselor's scope, and benzodiazepines are not a substitute treatment for depression.

35. C (Domain 4). Albert Roberts' model begins with a thorough biopsychosocial and lethality assessment, so the counselor knows whether the client is in immediate danger. A comes later, after rapport, problem identification, and exploring feelings and alternatives. B is the final stage.

36. B (Domain 3). An ABAB design records a baseline (A), introduces treatment (B), withdraws it (A), and reintroduces it (B). If the behavior changes each time, the case for the intervention is stronger. A stops after the first treatment phase. C staggers the start of treatment across settings without withdrawing it.

37. A (Domain 6). In telehealth, the laws of the state where the client is physically located generally apply. The counselor must hold authorization to practice there, such as a license or a Counseling

Compact privilege to practice. B ignores the client's location. C is wrong, because licensure rules apply to every form of counseling, including email.

38. B (Domain 1). The Council for Accreditation of Counseling and Related Educational Programs (CACREP) accredits counselor education programs. A, the National Board for Certified Counselors, offers national certification (the NCC) and develops the NCE. C, the American Counseling Association, is the profession's largest membership association and publishes the ACA Code of Ethics.

39. A (Domain 2). Lethal means counseling, working with the client to reduce access to highly lethal methods during a period of risk, is a key part of safety planning. A collaborative approach, such as temporary storage with someone she trusts, is more likely to be followed. B ignores a serious risk factor. C is a demand rather than a collaborative plan, and it may damage trust and lead the client to stop being open.

40. B (Domain 4). A double-sided reflection captures both sides of the client's ambivalence, a core motivational interviewing skill. A is the righting reflex, which tends to increase resistance. C is a "why" question that can feel like blame.

41. C (Domain 6). Counselors must report reasonable suspicion of child abuse. They do not need proof. A goes beyond the counselor's role, because investigation belongs to child protective services. B could put the child at greater risk.

42. C (Domain 3). Measurement-based care uses outcome data to guide decisions, and a lack of progress calls for a collaborative review and revision of the plan. A ignores the data. B is premature and may leave the client without help.

43. A (Domain 5). CPT 90791 is the psychiatric diagnostic evaluation without medical services. B is a 45-minute (38 to 52 minutes) individual psychotherapy session. C is group psychotherapy.

44. A (Domain 1). Supervision is an ongoing, evaluative, hierarchical relationship that protects clients and serves a gatekeeping role for the profession. Consultation is a voluntary, nonhierarchical relationship focused on a specific professional issue, and the consultee is free to accept or reject the advice. B reverses the two, because gatekeeping belongs to supervision. C has no basis, since either relationship can address one case or many.

45. C (Domain 2). A criterion-referenced approach compares each score with a fixed standard, which is how passing scores on licensing exams work. A compares scores with those of a norm group. B compares a person's scores with their own other scores.

46. A (Domain 4). Edna Foa's prolonged exposure uses imaginal exposure, which means revisiting the trauma memory, and in vivo exposure, which means facing avoided situations in real life. B describes EMDR. C describes narrative therapy.

47. A (Domain 6). When informal resolution fails and the misconduct is serious, such as insurance fraud, counselors take further action by reporting to the appropriate board or ethics committee. B leaves serious harm unaddressed. C could breach confidentiality and is not an appropriate step.

48. B (Domain 3). A specific aftercare plan with a scheduled appointment and relapse prevention supports bridges the move from one level of care to the next. A breaks continuity. C is too vague to support recovery.

49. C (Domain 5). A warm handoff happens when the counselor directly connects the client with the new provider, which raises the chance the client follows through. A gives the client a name or number and leaves the next step to them. B is a consultation relationship between professionals.

50. B (Domain 1). The ACA Code treats self-care as part of professional responsibility, because burnout and impairment can reduce the quality of care and harm clients. A misses that link. C waits until harm has already occurred.

Score Analysis

Count your correct answers in each domain. Compare your domain percentages with one another to see where to focus next.

Domain question map

Domain	Question Numbers	Total	My Correct	My Percent
1. Professional Development & Counselor Self-Awareness	4, 10, 16, 24, 31, 38, 44, 50	8	___	___%
2. Intake & Assessment	2, 8, 12, 17, 22, 27, 33, 39, 45	9	___	___%
3. Treatment Planning & Continuity of Care	5, 14, 21, 29, 36, 42, 48	7	___	___%
4. Provision of Counseling Interventions	1, 6, 11, 15, 20, 25, 30, 35, 40, 46	10	___	___%
5. Indirect Client Care	9, 19, 26, 34, 43, 49	6	___	___%
6. Legal & Ethical Compliance	3, 7, 13, 18, 23, 28, 32, 37, 41, 47	10	___	___%
Total		50	___	___%

A practice percentage is not a scaled score, and no practice exam can predict your official result. The 2027 NCE reports a scaled score, with 360 as the passing point, and that scaled score cannot be converted to a percentage. Use this exam to compare your domains and to track your progress over time.

What to do next

1. **Add every miss and every correct guess to your weak-spot tracker**, noting the error type: knowledge gap, misread stem, overthinking, real-world habit, distractor trap, or confident but wrong.
2. **Return to the matching domain chapter** in NCE Exam Prep 2026–2027 for your two lowest domains, and reread the Quick Recall and Exam Traps boxes.
3. **Review your cheat sheets and flashcards** for the topics you missed, and retest yourself on them in three to five days.

One Small Request

If NCE Exam Prep 2026–2027 has helped you, please consider leaving an honest review on Amazon. To find it, search for the book's title. Share your real opinion, whatever it is. Reviews help other counseling graduates decide whether the book fits how they study.